



Division of Provider Services & Quality Assurance

Office of Long-Term Care

P.O. Box 8059, Slot S404, Little Rock, AR 72203-1437

Notice of Serious Incident

Case Number: 032032

Date of Incident: 08/11/2025

Date Received: 08/12/2025

Facility Name: United Methodist Children's Home Little Rock Campus

Incident Type: DPSQA - Incident & Accident - OLTC

Case Status: Under Review

DPSQA - Incident & Accident - OLTC

Report Description: [REDACTED] became upset after a peer reported to staff that he was picking at sores on his arm. He exited the cafeteria and sat near the cafeteria door. Without warning, [REDACTED] stood up, walked to the hallway door, and struck it with his fist. **Medical Assessment & Findings:** [REDACTED] was promptly assessed by the nurse, who noted rapid swelling, moderate discomfort, and reduced range of motion with stiffness in his right hand. [REDACTED] reported a prior fracture in the same hand several years ago. [REDACTED] was notified, and an x-ray of the right hand (three views) was ordered along with a medical consult. [REDACTED] guardian and the Program Director were also notified. **Diagnostic Results:** ClearView Imaging arrived within minutes, as they were already scheduled to x-ray another client. [REDACTED]

Follow-Up Actions: The guardian, Program Director, [REDACTED], and the Nurse Practitioner were informed of the results. [REDACTED] issued an order for [REDACTED] to be transported to the [REDACTED] for further assessment and treatment.

Arkansas State Agency

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 3005	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING: _____	(X3) DATE SURVEY COMPLETED 08/18/2025
NAME OF PROVIDER OR SUPPLIER UNITED METHODIST CHILDRENS HOME		STREET ADDRESS, CITY, STATE, ZIP CODE 2002 S FILLMORE ST LITTLE ROCK, AR 72214		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
- 000	Initial Comment The facility was not in compliance with Title 20, Chapter 10. Office of Long Term Care, Division of Provider Services and Quality Assurance, Department of Human Services Part 415. Psychiatric Residential Treatment Facility Providers. ELS 32032 ELS 32021	- 000		
-304 SS=D	20 CAR 417 Staffing ratios and required personnel (a) The Psychiatric Residential Treatment Facility shall have: (1) A physician currently licensed by the Arkansas State Medical Board who has experience in the practice of adolescent psychiatry; (2) A Director of Nursing or Nurse Manager currently licensed in Arkansas as a Registered Nurse; (3) A Clinical Director who has at least a master ' s degree in a human services field and is currently licensed in Arkansas as a mental health professional as recognized by Arkansas Medicaid; 19 (4) One (1) or more therapists having at least a master ' s degree in a human services field and who is currently licensed as a mental health professional as recognized by Arkansas Medicaid; and (5) A therapist assigned to each child who is responsible for assessments, treatment planning, and casework services. (b) Staff shall be at least twenty-one (21) years of age and have a high school diploma or equivalent.	-304		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

Nicole Hull, LMSW

Program Director

Arkansas State Agency

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-304	Continued From page 1 (c) The Psychiatric Residential Treatment Facility shall meet minimum licensure requirements for staffing, staff qualifications, and staffing ratios. (d)(1) The Psychiatric Residential Treatment Facility shall maintain the following minimum staffing ratios: (A) One (1) staff person per four (4) Residents, for a minimum ratio of (1:4), from 6:00 a.m. to 10:00 p.m.; and (B) One (1) staff person per eight (8) Residents, for a minimum ratio of (1:8), from 10:00 p.m. to 6:00 a.m., or 11:00 p.m. to 7:00 a.m., depending on age and need. (2) The staff, for purposes of this ratio, shall be (A) Staff dedicated to provide direct care in that particular milieu. (B) Awake at all times. (C) Provide visual check on each child at least every (15) minutes but no greater than twenty (20) minutes. (D) Provide constant visual on each Resident in common areas during waking hours. (E) Document all visual checks. (d)(3) The Psychiatric Residential Treatment Facility shall maintain a daily census report to include the child ' s name and room or building assignment. (e) A Psychiatric Residential Treatment Facility shall modify staffing patterns to fit their increased needs. (f) Staffing ratios listed above are a minimum standard. The Psychiatric Residential Treatment Facility shall have written policies and procedures that: (1) Demonstrate how the staffing pattern will be adjusted when necessary to	-304		

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-304	Continued From page 2 meet the individual needs and acuity of youth as those fluctuate over time; (2) Document how the Psychiatric Residential Treatment Facility continuously monitors the appropriateness of its staffing pattern to ensure the safety of both the Residents and the staff; (3) Shall include specific methods used by the Psychiatric Residential Treatment Facility to monitor metrics such as restraints and seclusions and other adverse incidents, and documentation of how the Psychiatric Residential Treatment Facility uses this monitoring to make ongoing decisions about staffing patterns; (4) Document how the Psychiatric Residential Treatment Facility continuously monitors the appropriateness of its staffing pattern to ensure that Residents receive appropriate, individualized care, treatment and therapeutic interactions; and (5) This documentation shall include specific methods used by the Psychiatric Residential Treatment Facility to monitor metrics such as clinical progress and outcomes, and documentation of how the Psychiatric Residential Treatment Facility uses this monitoring to make ongoing decisions about staffing patterns. (g)(1) Beds exceeding licensure and permit will be authorized only in cases of emergency. 20 (2) An emergency exists when it can be demonstrated that the Resident ' s health or safety would be placed in immediate jeopardy if relocation were not accomplished. (3) A fire, natural disaster such as a flood or tornado, or other catastrophic event that necessitates Resident relocation will be	-304			

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-304	Continued From page 3 considered an emergency. (4) The Office of Long-Term Care must be contacted for prior authorization of the overbed, and all authorizations must be in writing. Authority. Arkansas Code §§ 20-76-201 et seq., and 25-10-129. This ELEMENT is not met as evidenced by: Based on video review and interview, the facility failed to maintain staffing to meet the needs for 1 (Client #1) who was on one-to-one observations of three clients reviewed. A review of the video surveillance on 08/18/2025 at 9:30 AM was conducted with the Director of the Residential Treatment Center (DRTC) for the hallway where Client #1 punched the wall and injured their hand on 08/11/2025. During the observation of the video footage at 06:15:38 PM Client #1 walked out of the line of sight from Behavioral Instructor (BI) #1 who was assigned as Client #1's one to one staff. At 06:15:47 Client #1 struck the wall with a closed fist. At 06:16:30, BI #3 entered the hall escorting another student and did not engage or address Client #1. BI #3 walked through the hall at 06:16:39 again and did not address Client #1. At 06:18:41 Client #1 punched the concrete wall with their right hand in a closed fist. At 06:19:09 Client #1 turned toward the door and struck the door with their right hand. At 06:33:19 BI #3 spoke to Client #1 and led him out of sight of the video. On 08/18/2025 at 9:45 AM during an interview while observing the video the DRTC stated the staff were not where they needed to be to observe the incident and that they should have been in the hall with the client. The DRTC further	-304	-304 On 8/27/25, a Lead Behavioral Instructor Meeting was held. During this meeting, Lead BIs were reminded of the Observation Rounds Policy, with specific emphasis on the requirement for 1:1 supervision. Each Lead BI was provided with a quick reference handout to share and review with their assigned BIs on shift. All Behavioral Instructors (Bis) were provided with a copy of the in-service material regarding the procedures of 1:1 supervision on 8/30/25, 9/2/25, and 9/3/25. Attendance was documented and filed.	

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-304	Continued From page 4 stated there was nothing to prevent staff from entering the hall when the client moved as ratio would have been maintained. On 08/18/2025 at 3:49 PM, during an interview BI #1 stated they were assigned as the one-to-one staff for Client #1. BI#1 stated one to one with a client means they are together throughout the shift. A follow up interview was conducted on 08/18/2025 at 5:37 PM with BI#1 who stated the reason they did not follow Client #1 into the hallway was because there were 17 other clients. On 08/18/2025 at 5:26 PM, during an interview the Program Consultant (PC) stated Client #1 returned from an acute hospital stay and remained on one-to-one precautions for continued suicidal ideation. The PC stated when a client is one to one, goes to an acute stay, returns to one to one and is left out of the care of staff for approximately fourteen minutes they have failed to ensure their safety because staff should always be with them. The PC stated that all staff are trained on one-to-one precaution and what that means during on-the-job training prior to working directly with clients. The PC stated this training is called Observation Rounds and culminates in a test which is maintained in employee files. A review of the Observation Rounds training dated 10/16/2025 was reviewed in BH #1's file and reflected 100%. A review of the Observation Records dated 08/05/2025 through 08/11/2025 indicated Client #1 was placed on one-to-one observation on 08/05/2025. Client #1 went to an acute stay on 08/06/2025 and returned to the facility on 08/10/2025. Client #1 returned with the following	-304	-304 The Special Precaution Policy, which includes clear expectations for 1:1 supervision, has been posted in BI areas and by the time clock for daily visibility. Staffing Assignments: Daily staffing ratio sheets now clearly indicate which staff are responsible for 1:1 supervision, including coverage during breaks and shift changes. Supervisor Oversight: Shift supervisors are required to verify that 1:1 staff are present and actively engaged with the assigned client during shift rounds. Video Review Audits: Random weekly audits of video footage are conducted by the Unit Consultants when there are 1:1 and/or LOS (line of sight) precautions to ensure staff are maintaining line-of-sight supervision for clients on 1:1 status. Findings are maintained in a binder and are reviewed by the Program Director weekly.	9/4/2025

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NAME OF PROVIDER OR SUPPLIER

STREET ADDRESS, CITY, STATE, ZIP CODE

UNITED METHODIST CHILDRENS HOME

**2002 S FILLMORE ST
LITTLE ROCK, AR 72214**

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-304	Continued From page 5 precautions: Elopement precautions, suicide precautions, assault precautions, building restriction, finger food only, one to one, line of sight 24/7, spork watch, no sharps, 30-minute medication watch.	-304		